Section of Psychiatry

President R K Freudenberg MD

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One Hundred Years after John Conolly

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We are assembled to honour Dr John Conolly who died on March 5 1866 – one hundred years ago. It is therefore pertinent to ask what he accomplished that we should single him out for this homage.

His contemporaries saw him as the practical reformer of the treatment of the insane whose non-restraint system was worthy of a place alongside the great philanthropic movements for the poor and the oppressed like Wilberforce's fight against the slave trade and Howard's work for prison reform, and later on a par with Jenner's introduction of vaccination and Lister's of antisepsis in warding off and alleviating human suffering. The Earl of Shaftesbury, as chairman of the Lunacy Commissioners, when making a presentation to Conolly on his retirement from Hanwell Asylum in 1852 said: 'To understand . . . the remarkable merits of Dr Conolly, we must remember the state of things which prevailed in Lunatic Asylums some years ago. Nothing could have been more horrible than the treatment of lunatics . . . regarded as savage beasts . . . only to be coerced, and the Lunatic Asylum was worse than the prison. Now, all that is changed . . . and . . . we owe the noble position which this country occupies at the head of one great department of beneficence and civilization to . . . Dr Conolly' (Clark 1869). He added, significantly, 'I find that while the class of lunatics has been raised in the moral scale of existence, society has generally benefited'. For Sir George Paget (1866), Regius Professor of Physic at Cambridge, 'a pauper lunatic asylum, such as may now be seen in our English counties . . . is . . . the most blessed manifestation of true civilization that the world can present'.

These sentimental eulogies have obscured Conolly's medical standing. By abolishing chains and hobbles, strait waistcoats and handcuffs, he not only removed the outward symbols of oppression from the poor and mad, but also gave them the dignity and status of mentally sick patients. Only two years after Conolly had introduced non-restraint, one of the first resolutions passed by the founders of what became the Royal Medico-Psychological Association at their inaugural meeting in 1841, was 'That . . . the terms *Lunatic*, and *Lunatic Asylum*, be abandoned . . . and the terms *Insane Person*, and *Hospital for the Insane* be substituted'.

Few recognized the medical significance of this advance, the first in the history of the insane which made it meaningful to contrast the present with the past. 'It appeared to me that then only could the proper study of insanity begin', wrote Conolly (1856), 'the removal of restraints, and all violent and irritating methods of control, then first permitting the student to contemplate disorders of the mind in their simplicity, and no longer modified by exasperating treatment.' Rightly did Robertson (1867) hail Conolly as the founder of 'the English School of Psychological Medicine'; and the opening paragraph of the first number of the *Asylum Journal* (November 15 1853), later the *Journal of Mental Science*, now the *British Journal of Psychiatry*, justified its birth with: 'From the time when Pinel obtained . . . permission . . . to try the humane experiment of releasing from fetters some of the insane citizens chained to the dungeon walls of the Bicêtre, to the date when Conolly announced, that in the vast Asylum over which he presided, mechanical restraint in the treatment of the insane had been entirely abandoned, and superseded by moral influence, a new school of special medicine had been gradually forming'.

Conolly was born on May 27, 1794, in Lincolnshire. His father was an impoverished Irishman and his mother a Tennyson of the poet laureate's family. After a period of military service during the Napoleonic scare he married Eliza Collins, the daughter of a distinguished and noble naval officer, by whom he had three daughters and one son. In 1818 he followed his brother William into medicine and studied at Glasgow and Edinburgh where he graduated MD in 1821. In 1829 he became LRCP and in 1844 FRCP; in 1852 Oxford awarded him an honorary DCL.

The first part of Conolly's medical career lasted eighteen years and was spent in general and consultant practice interrupted by three years as the newly founded London University's first professor of medicine; the second started in 1839 when at the age of 45 he took up his appointment as resident physician to the Middlesex County Pauper Lunatic Asylum at Hanwell, now St Bernard's Hospital. From then his life becomes part of the history of psychiatry.

In the first period his energies were diffused over a wide field of medical, social and educational activities. 'Being a reformer by nature and a hearty liberal in politics', wrote his son-in-law Henry Maudsley (1866), 'he ardently devoted himself to every measure of progress.' At Stratford-on-Avon he founded a hospital and dispensary for the sick poor and published in 1824 in face of a local epidemic the second edition of his book extolling vaccination, the first having appeared from Chichester two years before. He served as alderman and mayor, was active in the Shakespeare circle and in 1835 organized the Shakespeare Monumental Committee to preserve the chancel roof, effigy and tomb (Hunter & Macalpine 1964). Not only the poet but the student of the human mind and heart attracted Conolly, and his last book, published in 1863, was 'A Study of Hamlet'.

When the promotors of London University, now University College London, were looking for a professor of medicine they chose Conolly, a public-spirited Dissenter and accomplished physician with a radical outlook and graceful pen. He took up his post in 1828. But the lack of a university hospital for teaching students, not compensated by the dispensary or outpatient clinic which he himself organized, internal strife fostered by an autocratic warden, refusal of the managers to pay a living wage so that professors were forced into private practice against Conolly's principle that they concentrate on teaching and research, all these led to bitter quarrels and ultimately his resignation. In his valedictory lecture on April 29, 1831, he said that it had been his ambition by 'surveying the relations of all parts of theoretical and practical medicine . . . to furnish to others . . . a point from which useful discoveries might take their origin' and 'to prosecute some original branches of practical investigation' himself. But such a 'great task . . . demands leisure, and some degree of freedom from common anxieties, and from the necessity of making exertions not immediately connected with it. None of these advantages have been enjoyed, or were likely to be enjoyed by myself as professor here; and I had, consequently, no prospect of succeeding in a noble design to which I would gladly have devoted my mind, and my time, and even my life' (Conolly 1832).

He returned to the Midlands, settled at Warwick, aided Sir Charles Hastings to found in 1832 the Provincial Medical and Surgical Association, now the British Medical Association, edited with Sir John Forbes and Alexander Tweedie the great four-volume 'Cyclopædia of Medical Practice' (1833–5), and helped to found and edit the *British and Foreign Medical Review* in 1836. In 1838–9 he spent some months lecturing on medicine at Birmingham.

In 1860 he wrote 'My interest in the insane . . . began early, and became increased as years advanced'. In 1816 he had returned from Army service to Hull where Sir William Charles Ellis (the first psychiatrist to be knighted for services to the specialty) had founded the Refuge for the Insane inspired by the example of the nearby Retreat at York. In 1831 Ellis was appointed physician to Hanwell when it opened and there Conolly followed him eight years later. Conolly continued: 'In the first years of my medical studies, my thoughts . . . were more consistently directed to mental phenomena, and especially to those of minds in a disordered state, by an accidental visit to the old Lunatic Asylum of Glasgow [in 1819] . . . under the general direction of Dr [Robert] Cleghorn. . . . The impression made in that and several other visits . . . has never been effaced.' In May 1821 he met Samuel Tuke through his fellow student Thomas Hodgkin and in August presented his MD thesis 'De Statu Mentis in Insania et Melancholia'. Like a signpost to the future it carried a motto from Pinel and the germs of many later ideas. To Tuke's 'Description of the Retreat' (1813) he gave pride of place in treatment: 'Although in most mental illnesses it is necessary first to look to the condition of the body, and in some this is all that is required, nevertheless their causes and symptoms frequently demonstrate that our efforts ought to be concentrated on the mind. I do not

hesitate to assert that more insane persons would be cured if moral treatment were better understood and administered in time'.

He had to wait eighteen years for the opportunity to look after the insane himself. In the interval he became, from 1824 to 1828, inspecting physician to the lunatic houses of Warwickshire, an appointment under the Act for Regulating Madhouses (1774) which laid down that two magistrates accompanied by a physician were to inspect the private asylums licensed by Quarter Sessions.

His first psychiatric paper was a review article in the *London Medical Repository* of 1827 (a journal he had helped to edit since 1825) of Bayle's and Calmeil's now classic texts published at Paris the previous year on what became known as general paralysis of the insane. In it he introduced to English readers the concept, natural history and clinical features of this disease and in his Croonian Lectures (1849) gave the first account in this country of a large series of personally observed cases. The disease has a unique position in psychiatry because it was the first in which a mixed neuropsychiatric condition could be traced to a single cerebral pathology. Before then searchers for a neuropathology of 'madness' had been repeatedly mortified by the apparent paradox that gross mental symptoms existed without cerebral pathology, while gross cerebral pathology was often found in patients without mental symptoms. Here both were combined, a fact which at the end of the nineteenth century so impressed Kraepelin that he used it as his model for dementia praecox. Conolly concluded: 'At the present stage of our investigation into diseases of the mind, and their physical causes, what we want is facts, recorded with a scrupulous adherence to truth. It must be after their production and accumulation, far beyond what we at present possess, that we can feel at all justified in deducing positive conclusions regarding the connexion of particular forms of madness with specific morbid alterations'.

In the same year, 1827, in the prospectus of the new London University and again in his Introductory Lecture, delivered October 2, 1828, Conolly spoke of the need to teach psychiatry in a medical course. He said: '*Mental Disorders* . . . than which none are more perplexing to the practitioner, or so often involve him in circumstances of heavy responsibility, will be very fully investigated, and every opportunity taken to make the student practically acquainted with their forms and management'. However, in the turmoil to which reference has already been made, his plan came to nothing. Maudsley's (1866) comment shows how far Conolly was ahead of his time: 'Only last year [1865] the Senate of the University of London, whose serious consideration of the subject I was able to obtain, was compelled to refrain from issuing compulsory regulations enforcing a clinical knowledge of insanity from candidates for degrees, solely by reason of the want of adequate means of clinical instruction. . . . When we reflect on how little has been done now . . . we may better appreciate the value of the attempt made more than thirty years ago.' It was this disappointment which led Conolly to publish from his professorial chair in 1830 his first psychiatric book 'The Indications of Insanity', significantly subtitled 'With suggestions for the better protection and care of the insane'. He laid down four principles for a national mental health service including domiciliary care: (1) That insanity itself was not sufficient reason for confinement. (2) That every insane person should become a child of the state and every asylum its property under central control. (3) That every asylum should be a school of instruction for medical students, and a place of education for male and female attendants. (4) That every asylum should concern itself with the welfare of the mentally ill in its community in collaboration with general practitioners.

In June 1839, one year after the resignation of Sir William Ellis, Conolly started his life's work as resident physician at Hanwell, a post he held for five years. He drew on his experience here in all his later writings and the first group of these, his first four Reports (1842), was accorded by the magistrates of Middlesex the unique honour of being reprinted in book form. Within four months he had abolished all personal restraint and was learning the clinical lessons of this epoch-making advance. 'The great principle of the non-restraint system', he wrote, 'is to exclude all hurtful excitement. . . . On this principle it abolishes mechanical restraint and also . . . regulates every word, look, and action of all who come in contact with the insane.'

But it must not be thought that it was received with open arms. On the contrary, to quote Maudsley again: 'In the face of much prejudice and many obstacles, the direct opposition of open enemies, the doubts of friends, and machinations of secret ill-wishers eager for its failure, he steadily applied the humane system of treatment throughout the asylum, which then contained 850 patients suffering from every form of acute and chronic insanity. . . . The reputation which he had when he became physician to the asylum, its close proximity to London, where his work could

not fail to excite attention, his great literary facility and long experience of the press, combined to force the question into general notice.... He... enforced a certain improved system of practice... expounded the humane and scientific theory of it, and set forth eloquently the wide-reaching and beneficial consequences of its adoption. He not only made the hitherto obscure movement a world-known success, but he made reaction to it impossible.'

After an experience of three years Conolly (1842) was able to establish once and for all 'that the management of a large Asylum is not only practicable without the application of bodily coercion to the patients, but that, after the total disuse of such a method of control, the whole character of an asylum undergoes a gradual and beneficial change'. So began also the asylum era of the nineteenth century from which much of what we know and think we know about psychiatry today derives. In his second report Conolly had detailed new approaches to treatment which had developed in place of the old restraints and to which we owe the concept of the therapeutic community: 'The application of medical science is not limited in any disease to the administration of drugs... least of all in diseases of the nervous system.... For... mental treatment the opportunities are constant; and the materials exist in the general arrangements of the house... the regulation of the diet, the exercise, the hours of rest, the occupations, the amusements, the dress, and conduct, become of wide application and extreme importance. These matters, well arranged, become general medicines.' 'The new system is a positive treatment', wrote Professor Griesinger (1867) of Berlin who had first been sceptical, 'not a mere leaving the patient to himself.'

In lectures, articles and books Conolly distilled his experience. Of these the most important are 'The Construction and Government of Lunatic Asylums and Hospitals for the Insane' (1847) which until this century remained the handbook of the administrative psychiatrist; and 'The Treatment of the Insane without Mechanical Restraints' (1856). Although he did not claim to cure but rather to place patients in the best circumstances for recovery, he found a remarkable increase in recovery rate up to a half or even two-thirds of all recent cases. This made him the first to appreciate fully how grim had been the effect of the old system on the manifestations of mental illness: 'Insanity... undergoes great... modifications; and the wards of lunatic asylums no longer illustrate the harrowing description of their former state. Mania, not exasperated by severity, and melancholia, not deepened by the want of all ordinary consolations, lose the exaggerated character in which they were formerly beheld.' He found also that the old standard physical treatments were now not only no longer necessary but indeed harmful: 'The physician... need not be ashamed to acknowledge with regard to mental phenomena... [that his] office is assuming, in these times, a higher character in proportion as he ceases to be a mere prescriber of medicines'. In their place he substituted the therapeutic regimen of the hospital centred on the mental nurse who owes her office and duties to him: 'The physician who justly understands the non-restraint system well knows that the attendants are his most essential instruments.... They may often be considered, indeed, his best medicines.... He entrusts them with the happiness, by day and by night, of all the patients under their especial care. To control the violent without anger; to soothe the irritable without weak and foolish concessions; to cheer and comfort the depressed; to guard the imbecile and the impulsive, and to direct all.' Only when all restraint had been abolished could the lunatic gain the dignity of a sick human being: 'when the patient is tied up, all regard for him ceases', he wrote; and only then could the varieties of mental illness be demonstrated in their natural form: 'At Hanwell, clinical teaching was commenced in 1842.... Scenes of general confusion and agitation, opposed to the possibility of study had become rare; the wards were tranquil, the patients cheerful... and the actual state of the minds of the insane was in most cases easily displayed to the learner.' Conolly also pursued ætiological studies within the limits of contemporary knowledge and tried to give them a statistical basis by organizing a system of case records and post-mortem examinations. But it was the lunatic, the insane patient as a person, who was the central figure in Conolly's scheme and less the diseases which made him so.

In 1844 ill-health forced Conolly to resign his post and he became in turn consulting and visiting physician until 1852 when his official connexion with Hanwell ceased. In 1847 he joined the Reverend Dr Andrew Reed in providing the same advantageous conditions for the mentally subnormal and helped to found at Highgate the first 'Asylum for Idiots'. In 1843 and 1851 he served as chairman of the Association of Medical Officers of Asylums and in 1858 as president of the Medico-Psychological Association.

To ensure that the abuses of old, which had taken so long and so hard a fight to abolish, could never prevail again, and to provide the

benefits of the new psychiatry for all the insane, including those still confined in workhouses and elsewhere, there was established a complex legal system of certification, inspection and control starting in 1845 with the Act for the Regulation of the Care and Treatment of Lunatics. This supporting structure gradually outlived its usefulness and now that it is no longer needed it has proved possible to dismantle it by the greatest single advance in organized psychiatry since Conolly's, namely the Mental Health Act of 1959. This step could never have been contemplated without the groundwork he laid; for the abolition of legal safeguards which not only makes it possible for the first time to treat the mentally ill like any patient, but has enabled patients to leave crowded mental hospitals and return to the community supported by community care, could only safely be attempted after Conolly had established within the asylum that the mentally ill could be and demonstrably were treated as human beings. How naturally this development followed on Conolly's work was sensed by Stallard (1869), one of his pupils: 'Now . . . the question may properly be asked, whether . . . we cannot recur, in some degree, to the system of home care and home treatment; whether, in fact, the same care, interest, and money which are now employed upon the inmates of our lunatic asylums, might not produce even more successful and beneficial results if made to support the efforts of parents and relations in their humble dwellings. . . . Such a mode of treatment would form a fitting extension of the non-restraint system.'

It is this new era of psychiatry we have now entered, and that we do so confidently is the most fitting tribute to Conolly's work. We take with us two fundamental tenets of psychiatric practice which he taught the world: symptom tolerance, and the distinction between treatment and restraint.

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The successful application of advances in the understanding of mental disorder and the acceptance of treatment depend upon the confidence of the public and the mentally ill in the probity of psychiatrists, the humanity of mental nurses and the reliability of institutions in which they may be treated. When society fails to grant the rights and freedoms traditionally accorded to the physically ill, to those who are mentally sick, their treatment must be hindered. The story of the achievement in this country for the mentally disordered, of a status almost comparable to that of the physically sick, is an important chapter in the history of psychiatry. It had its origins in the writings and works of Dr John Conolly and opens with his book 'Indications of Insanity' published in 1830. The important chapter is the tenth, 'Application of the enquiry to the duties of medical men when consulted concerning the state of a patient's mind', in which he sets out his views on the doctor's duty to the insane patient: 'We should present ourselves to him with the same good intention with which we approach the bedside of the patient with a fever.' He must not allow 'the agitation of those who make the application to disturb (his) the practitioner's judgment', but base his decision on his own examination since his primary obligation is to the patient. 'Against this forgetfulness of his office and duty, the best security will be found in such previous study of the subject as I have presumed to recommend.' When proof of mental disorder is difficult to elicit, the physician has no more right to restrain the mentally disordered 'than to imprison a man for being shortsighted or a little lame in one leg'.

Conolly was not moved so much by good intention or humanitarian concern, but by the belief that mental disorder was a disease to be studied and treated, despite a paucity of clinical knowledge. And he was well aware of the potentially harmful or beneficial effect on the patient of his social environment.

Conolly was a liberal and humane man. He was opposed to capital punishment, an advocate of the education of women and did not fear jibes of 'morbid philanthropy and visionary benevolence'. He considered that 'the safety and liberty of men was not lightly to be trifled with'. His belief in the need to reduce the restraints imposed on the mentally ill – not only mechanical restraints 'but confinement, deprivation of authority and control over property' – was determined not by an unscientific humanism, nor by the recognition